

PEDIATRIC GASTROENTEROLOGY — Revision Table

Bold = high-yield. Dr Heersh Raof Saeed.

ABDOMINAL PAIN	
Type	Key points
Visceral	Bilateral unmyelinated; dull, poorly localized, midline ; ischaemia/inflammation/stretch
Parietal (somatic)	Parietal peritoneum; sharp, intense, localized ; worse on movement/cough
Referred	Distant site; pneumonia → abdominal pain (T9)
Acute <2yr	Surgical: malrotation, volvulus, intussusception, NEC, incarcerated hernia ; medical: GE, UTI
Chronic >2yr	Appendicitis, Meckel's; non-specific pain, IBD, mesenteric lymphadenitis
Commonest medical cause	Gastroenteritis (rota/noro/adeno; E. coli, Salmonella, Shigella, Campylobacter)
Analgesia	Use as required — doesn't affect diagnostic accuracy

SPECIFIC CAUSES	
Condition	Key points
Infantile colic	Up to 10% infants, <6mo ; scream + draw knees up; evening, relieved by flatus; normal exam
Mesenteric adenitis	Post-URTI/adenovirus ; mimics appendicitis; shifting tenderness , higher temp
Appendicitis	Central → RIF pain; Rovsing's, psoas, obturator signs, McBurney's ; neutrophil leucocytosis + lymphopenia; retrocaecal = poor signs
HSP	IgA vasculitis (skin + kidney) ; young children; self-limiting; kidney damage ~1%

VOMITING & GERD	
Topic	Key points
Vomiting colour	Proximal obstruction (pyloric/IHPS) = non-bilious; distal obstruction = bilious ; CNS = non-bilious
GER vs GERD	GER physiological; GERD pathologic (erosive oesophagitis → hiatal herniation)
Diagnosis	pH monitoring (acid reflux + symptom association), UGI series, endoscopy + biopsy
Conservative (infant)	Thickened formula, non-prone positioning, hypoallergenic trial
Conservative (older)	Avoid large meals/caffeine/spicy, weight loss, no tobacco

CONSTIPATION	
Topic	Key points
Definition	Hard stool, difficulty, every 3 days, ≥2 weeks
Organic	Hirschsprung, spina bifida, hypothyroidism, anal stenosis, IBD, CF, lead
Functional	Commonest; painful defecation → stool withholding; soiling (overflow)
Diagnosis	Clinical; refractory → barium enema + anorectal manometry (rule out Hirschsprung)
Bristol chart	Type 1 (hard lumps) → 7 (liquid); 3–4 normal
Treatment	Education + bowel training; disimpaction (enema/PEG); softening (lactulose); maintenance laxatives